

Impetigo (Infected Sore) - Female (Child)

Algorithm 1196 | Child (0-17 years) | Gender at birth: Female

Definition

- Impetigo assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Female.

Initial assessment

1. [LOCATION] Where are the sores?
2. [DURATION] How long have they been present?
3. [YES_NO] Has this happened before in the last year?
4. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Disposition level 50

Is this a localized rash or redness without honey-colored crusts, weeping sores, or suspected impetigo?

The localized-rash family is not generated. Arrange clinical review through the Qatar route approved by governance (GOVERNANCE_REQUIRED).

1. Disposition level 100

Is there confusion, collapse, abnormal breathing, pale/blue/mottled skin, rapidly spreading painful redness, blistering or skin loss, severe pain, swelling near the eye or airway, or a newborn or infant who is feverish or very unwell?

Universal emergency rule-out added ahead of the impetigo guidance itself - impetigo itself is rarely an emergency, but a severe, spreading skin infection with systemic illness needs urgent care.

1. Disposition level 50

Without an emergency feature, is the patient under 1 year, pregnant or breastfeeding with breast lesions, diabetic or immunocompromised, are lesions near the eye, widespread, recurrent, worsening, or not responding, or is there a safeguarding concern?

NHS.UK guidance lists these as reasons for clinical review rather than pharmacist self-care. Impetigo is very infectious; the Qatar attendance route must be approved by clinical governance before use (GOVERNANCE_REQUIRED).

1. Disposition level 15

Is this a straightforward case (child 12+ months, no breastfeeding sores, no weakened immune system) with none of the features above?

NHS.UK guidance supports pharmacist assessment and treatment for straightforward impetigo; the applicable Qatar service route requires clinical-governance approval (GOVERNANCE_REQUIRED).

Advice

Emergency skin-infection precautions

- Call Qatar 999 now, do not allow self-driving, and do not squeeze, pierce, or apply caustic products. Follow the call-handler's instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: confusion, collapse, breathing change, severe pain, blistering, or rapidly spreading infection.

Governance-approved impetigo review

- Use the Qatar governance-approved in-person pathway and call ahead for infection-control advice. Infant, pregnancy, breastfeeding, immune, diabetes, eye-area, medicine, and safeguarding decisions require clinician review.
- Call Back If: worsening symptoms, fever, eye swelling, pain, or rapid spread.

Pharmacy self-care for impetigo

- A pharmacist may assess and treat straightforward impetigo where permitted; the applicable Qatar service route requires clinical-governance approval (GOVERNANCE_REQUIRED). Wash affected areas with soap and water, wash hands frequently (especially before/after applying cream), and wash bedding/towels at high temperature. Do not touch or scratch sores. Stay away from work, school, or nursery until no longer contagious (48 hours after starting treatment, or once patches dry and crust over without treatment). Don't share towels or prepare food for others.
- Call Back If: not improving with treatment; spreading or worsening.

First aid

- Call Qatar 999 now, do not allow self-driving, and do not squeeze, pierce, or apply caustic products. Follow the call-handler's instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Impetigo", <https://www.nhs.uk/conditions/impetigo/> (page last reviewed 06 June 2024)

Provenance

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